

When should I suspect type 2 diabetes in a child or young person?

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- **Consider a diagnosis of type 2 diabetes if a child or young person presents with persistent hyperglycaemia that may be accompanied by clinical features.**
 - Possible clinical features of type 2 diabetes include:
 - Symptoms such as polydipsia, polyuria, nocturia or enuresis, blurred vision, unexplained weight loss, recurrent infections, and tiredness. Note: these may be mild or absent.
 - Behavioural changes such as reduced school performance and/or impaired growth.
 - Signs such as acanthosis nigricans (a skin condition causing dark pigmentation of skin folds, typically the axillae, groin, and neck), which suggests insulin resistance.
 - The presence of [risk factors](#) ([/topics/diabetes-type-2/background-information/risk-factors/](#)).
 - Persistent hyperglycaemia is defined as:
 - Fasting plasma glucose level of 7.0 mmol/L or more.
 - Random plasma glucose of 11.1 mmol/L or more in the presence of symptoms or signs of diabetes.
 - HbA1c of 48 mmol/mol (6.5%) or more.
 - Be aware that a diagnosis of type 2 diabetes is more likely if there are:
 - No additional features of type 1 diabetes. See the CKS topic on [Diabetes – type 1](#) ([/topics/diabetes-type-1/](#)) for more information.
 - No clinical features of [other types of diabetes](#) ([/topics/diabetes-type-2/background-information/definition/](#)), such as monogenic diabetes or diabetes secondary to another condition or disease.
- **If a diagnosis of type 2 diabetes is suspected in a child or young person:**
 - Arrange immediate same-day referral to a multidisciplinary paediatric diabetes team to confirm the diagnosis and provide immediate care.

Basis for recommendation

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